A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

11 May 2026

Dr Cathryn Liebau
The Crane General Practice

Dear Dr Liebau,

Re: Dr Simon Baylis
DOB: 24/12/1988 Phone: 0439081039
Address: 164 Mckean Street, FITZROY NORTH, VIC, 3068
Email: sbaylis77@gmail.com

Summary

- **Post-operative review for Crohn's disease (penetrating and stricturing phenotype).**
- **Commencing ustekinumab as post-operative prophylaxis.**
- **Plan for surveillance via faecal calprotectin, colonoscopy in two months, and MRI in October.**
- **No action required by you.**

I had the pleasure of conducting a review with Simon and his wife Jessica following his recent surgery for Crohn's disease, performed by my excellent collegue Mr Michael Johnston. He is now one month post-operative and has returned to work. I am pleased to report that he is systemically well, denying any fevers or sweats, and is maintaining his weight.

We spent a significant portion of the consultation discussing the strategy for post-operative prophylaxis. Simon's Crohn's phenotype is somewhat aggressive, characterized by both stricturing and penetrating disease, the latter evidenced by the identification of an abscess. Standard practice in this setting often involves anti-TNF agents such as adalimumab or infliximab. However, we have opted to commence ustekinumab, with his induction infusion scheduled for tomorrow.

My rationale for this, which Simon supports, is based on the superior safety profile of ustekinumab, particularly regarding the lower risk of skin malignancy. While anti-TNF data is more robust in the post-operative setting, ustekinumab is likely to be similarly effective in preventing recurrence at the anastomosis. If we detect early recurrence on surveillance, we will pivot to adalimumab.

Impression and Plan

I have provided Simon with a pathology request for baseline bloods and a faecal calprotectin. Our monitoring strategy will involve serial calprotectin measurements followed by a surveillance colonoscopy in two months' time (marking three months post-surgery). I then plan to perform an MRI in October to further evaluate the integrity of the resection and screen for early recurrence. We will review the necessity of escalating the ustekinumab dose to a four-weekly interval based on those findings. I will review him again following his colonoscopy.

All appointments and enquires
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East Melbourne VIC 3002
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Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Mr Michael Johnston

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